Contents

heart failure is due to reduced ejection fraction, who are currently treated with a beta-blocker licensed for heart failure		
HF007. The percentage of patients with a diagnosis of heart failure on the register, who have had a review in the preceding 12 months, including an assessment of functional capacity and a review of medication to ensure medicines optimisation at maximal tolerated doses	7	50-90%
Hypertension (HYP)	**Points**	**Thresholds**
Ongoing management		
HYP008. The percentage of patients aged 79 years or under with hypertension in whom the last blood pressure reading (measured in the preceding 12 months) is 140/90 mmHg or less (or equivalent home blood pressure reading)	38	40-85%
HYP009. The percentage of patients aged 80 years or over, with hypertension, in whom the last blood pressure reading (measured in the preceding 12 months) is 150/90 mmHg or less, (or equivalent home blood pressure reading)	14	40-85%
Stroke and transient ischaemic attack (STIA)	**Points**	**Thresholds**
Ongoing management		
STIA007. The percentage of patients with a stroke shown to be non-haemorrhagic, or a history of TIA, who have a record in the preceding 12 months that an anti-platelet agent, or an anti-coagulant is being taken	4	57–97%
STIA014. The percentage of patients aged 79 years or under, with a history of stroke or TIA, in whom the last blood pressure reading (measured in the preceding 12 months) is 140/90 mmHg or less (or equivalent home blood pressure reading)	8	40-90%

DM020. The percentage of patients with diabetes, on the registers, without moderate or severe frailty in whom the last IFCC-HbA1c is 58 mmol/mol or less in the preceding 12 months	17	35-75%
DM021. The percentage of patients with diabetes, on the register, with moderate or severe frailty in whom the last IFCC-HbA1c is 75 mmol/mol or less in the preceding 12 months	10	52-92%
DM034. The percentage of patients with diabetes aged 40 years and over, with no history of cardiovascular disease and without moderate or severe frailty, who are currently treated with a statin (excluding patients with type 2 diabetes and a CVD risk score of <10% recorded in the preceding 3 years) or where a statin is declined or if clinically unsuitable, another lipid-lowering therapy	4	50-90%
DM035. The percentage of patients with diabetes and a history of cardiovascular disease (excluding haemorrhagic stroke) who are currently treated with a statin or where a statin is declined or if clinically unsuitable, another lipid-lowering therapy	2	50-90%
Asthma (AST)	**Points**	**Thresholds**
Initial diagnosis		
AST012. The percentage of patients with a new diagnosis of asthma on or after 1 April 2025 with a record of an objective test between 3 months before or 3 months after diagnosis	15	45–80%
Ongoing management		
AST007. The percentage of patients with asthma on the register, who have had an asthma review in the preceding 12 months that includes an assessment of asthma control, a	20	45–70%

Blood pressure (BP)	Points	Thresholds
BP002. The percentage of patients aged 45 or over who have a record of blood pressure in the preceding 5 years	15	50–90%
Smoking (SMOK)	**Points**	**Thresholds**
Records		
SMOK002. The percentage of patients with any or any combination of the following conditions: CHD, PAD, stroke or TIA, hypertension, diabetes, COPD, CKD, asthma, schizophrenia, bipolar affective disorder or other psychoses whose notes record smoking status in the preceding 12 months	25	50–90%
Ongoing management		
SMOK004. The percentage of patients aged 15 or over who are recorded as current smokers who have a record of an offer of support and treatment within the preceding 24 months	12	40–90%
Vaccination and Immunisations (VI)	**Points**	**Thresholds**
VI001. The percentage of babies who reached 8 months old in the preceding 12 months, who have received at least 3 doses of a diphtheria, tetanus and pertussis containing vaccine before the age of 8 months	18	89-96%
VI002. The percentage of children who reached 18 months old in the preceding 12 months, who have received at least 1 dose of MMR between the ages of 12 and 18 months	18	86-96%
VI003. The percentage of children who reached 5 years old in the preceding 12 months, who have received a reinforcing dose of DTaP/IPV and at least 2 doses of MMR between the ages of 1 and 5 years	18	81-96%

AF008. Percentage of patients on the QOF Atrial Fibrillation register and with a CHA2DS2- VASc score of 2 or more, who were prescribed a direct-acting oral anticoagulant (DOAC), or, where a DOAC was declined or clinically unsuitable, a Vitamin K antagonist	12	70-95%

AF – rationale for inclusion of indicator set

i. AF is the most common heart rhythm disorder, affecting approximately 2% of the adult population, and estimates suggest its prevalence is increasing. AF causes palpitations and breathlessness in many people, but it may also be asymptomatic and therefore go undetected. Left untreated, AF is a significant risk factor for stroke: it is estimated that it is responsible for approximately 20% of all strokes and is associated with increased mortality and significant morbidity. Men are more commonly affected than women. AF prevalence increases with age and in association with heart disease, diabetes, obesity and hypertension.

AF006 (based on NICE IND127)

AF006 Rationale

i. The NICE guideline on atrial fibrillation[3] recommends that people with symptomatic or asymptomatic paroxysmal, persistent or permanent AF, atrial flutter or a continuing risk of arrhythmia recurrence after cardioversion back to sinus rhythm should have an assessment of their stroke risk using the CHA_2DS_2-VAS_c risk assessment tool.

ii. The CHA2DS2-VASc system scores one point, up to a maximum of nine, for each of the following risk factors (except previous stroke or TIA, or age ≥75 which scores double, hence the '2'):

- C: congestive HF (one point)
- H: hypertension (one point)
- A_2: age 75 or over (two points)
- D: diabetes mellitus (one point)
- S_2: previous stroke or TIA or thromboembolism (two points)
- V: vascular disease (e.g. PAD, MI, aortic plaque) (one point)
- A: age 65-74 years (one point)

[3] NICE NG196 Atrial fibrillation (2021) http://www.nice.org.uk/Guidance/NG196

CHD015 (based on NICE IND241)

CHD015 Rationale

i. This indicator measures the intermediate outcome of a blood pressure of 140/90 mmHg or less in people aged 79 years or under with CHD. The aim is to promote secondary prevention of cardiovascular disease through satisfactory blood pressure control. This may be achieved through lifestyle advice or drug therapy.

ii. This indicator has been updated for 2024/25 to align with updated NICE guidance which recommends differential blood pressure control targets for readings taken with Home Blood Pressure Monitoring (HBPM) and readings taken in a clinical setting. A target for stage 1 hypertension of 140/90mmHg taken in a clinic corresponds to an HBPM target of 135/85 mmHg.

CHD015 Reporting and verification

i. See indicator wording for requirement criteria.

CHD016 (based on NICE IND242)

CHD016 Rationale

i. This indicator measures the intermediate outcome of a blood pressure of 150/90 mmHg or less in people aged 80 years and over with coronary heart disease, as recommended by the NICE clinical guideline for hypertension[6].

ii. This indicator has been updated for 2024/25 to align with updated NICE guidance which recommends differential blood pressure control targets for readings taken with Home Blood Pressure Monitoring (HBPM) and readings taken in a clinical setting. A target of 150/90mmHg taken in a clinic corresponds to an HBPM target of 145/85 mmHg.

CHD0016 Reporting and verification

i. See indicator wording for requirement criteria.

[6] NICE NG136 (2019, updated 2023) Hypertension in adults http://www.nice.org.uk/guidance/ng136

HF007 (based on NICE IND195)

HF007 Rationale

i. Regular review is associated with improvement in quality of life and a reduction in the need for urgent hospitalisation. NICE guideline NG106 recommends short monitoring intervals (days to 2 weeks) if the clinical condition or medication has changed and 6-monthly for people with stable heart failure.

ii. More detailed monitoring will be needed if the person has significant comorbidity or if their condition has deteriorated since the previous review, with consideration for individualised care for frailty and palliative and end of life care.

HF007 Reporting and verification

i. See indicator wording for requirement criteria.

3.5 Hypertension (HYP)

Indicator	Points	Thresholds
Ongoing management		
HYP008. The percentage of patients aged 79 years or under with hypertension in whom the last blood pressure reading (measured in the preceding 12 months) is 140/90 mmHg or less (or equivalent home blood pressure reading)	38	40-85%
HYP009. The percentage of patients aged 80 years or over with hypertension in whom the last blood pressure reading (measured in the preceding 12 months) is 150/90 mmHg or less (or equivalent home blood pressure reading)	14	40-85%

HYP008 (based on NICE IND239)

HYP008 Rationale

i. This indicator measures the intermediate outcome of a blood pressure of 140/90 mmHg or less in people aged 79 years or under with hypertension. Its intent is to promote the primary and secondary prevention of cardiovascular disease through satisfactory blood pressure control. The intermediate outcome can be achieved through lifestyle advice or the use of drug therapy.

ii. This indicator was updated in 2024/25 to align with updated NICE guidance which recommends differential blood pressure control targets for readings taken with Home Blood Pressure Monitoring (HBPM) and readings taken in a clinical setting. A target for stage 1 hypertension of 140/90mmHg taken in a clinic corresponds to an HBPM target of 135/85 mmHg.

HYP008 Reporting and verification

i. See indicator wording for requirement criteria.

HYP009 (based on NICE IND240)

HYP009 Rationale

i. The NICE guideline for hypertension[17] recommends that patients aged 80 years and over with hypertension should be treated to a target blood pressure below 150/90 mmHg. It also recommends that this group of patients should be offered the same antihypertensive drug treatment as people aged 55-80 years, taking into account any co-morbidities.

ii. Where people have had a lower treatment target before the age of 80 years their treatment should continue and not be adjusted or down titrated. There is an important distinction between continuing long term and well tolerated treatment in people aged 80 years and older and starting blood pressure lowering therapy at this age.

iii. This indicator was updated in 2024/25 to align with updated NICE guidance which recommends differential blood pressure control targets for readings taken with Home Blood Pressure Monitoring (HBPM) and readings taken in a clinical setting. A target of 150/90mmHg taken in a clinic corresponds to an HBPM target of 145/85 mmHg.

HYP009 Reporting and verification

i. See indicator wording for requirement criteria.

[17] NICE NG136 (2019, updated 2023) Hypertension in adults. http://www.nice.org.uk/guidance/ng136

ii. The NICE guideline on hypertension[20] recommends drug therapy in people aged 79 years and under with stage 1 hypertension and cardiovascular disease. Antihypertensive drug treatment is recommended for people of any age with stage 2 hypertension.

iii. This indicator was updated in 2024/25 to align with updated NICE guidance which recommends differential blood pressure control targets for readings taken with Home Blood Pressure Monitoring (HBPM) and readings taken in a clinical setting. A target for stage 1 hypertension of 140/90mmHg taken in a clinic corresponds to an HBPM target of 135/85 mmHg.

STIA014 Reporting and verification

i. See indicator wording for requirement criteria.

STIA015 (based on NICE IND244)

STIA015 Rationale

i. This indicator measures the intermediate outcome of a blood pressure of 150/90 mmHg or less in people aged 80 years and over with a history of stroke or TIA. The aim of treating people to this target is to promote secondary prevention of vascular events through satisfactory blood pressure control.

ii. This indicator was updated in 2024/25 to align with updated NICE guidance which recommends differential blood pressure control targets for readings taken with Home Blood Pressure Monitoring (HBPM) and readings taken in a clinical setting. A target of 150/90mmHg taken in a clinic corresponds to an HBPM target of 145/85 mmHg.

STIA015 Reporting and verification

i. See indicator wording for requirement criteria.

3.7 Diabetes mellitus (DM)

Indicator	Points	Thresholds
Ongoing management		

[20] NICE NG136 (2019, updated 2023) Hypertension in adults. http://www.nice.org.uk/guidance/ng136

DM034. The percentage of patients with diabetes aged 40 years and over, with no history of cardiovascular disease and without moderate or severe frailty, who are currently treated with a statin (excluding patients with type 2 diabetes and a CVD risk score of <10% recorded in the preceding 3 years) or where a statin is declined or if clinically unsuitable, another lipid-lowering therapy	4	50-90%
DM035. The percentage of patients with diabetes and a history of cardiovascular disease (excluding haemorrhagic stroke) who are currently treated with a statin or where a statin is declined or if clinically unsuitable, another lipid-lowering therapy	2	50-90%

DM – rationale for inclusion of indicator set

i. Diabetes mellitus (DM) is a common endocrine disease. In 2023/24, there were approximately 3.7 million people living with diagnosed diabetes in England. Effective control and monitoring can reduce mortality and morbidity. Much of the management and monitoring of diabetes, particularly type 2 diabetes, is undertaken by the GP and members of the primary care team.

ii. Further information:

- NICE NG28 (2015, updated 2022) Type 2 diabetes in adults. http://www.nice.org.uk/guidance/NG28
- NICE NG19 (2015, updated 2019). Diabetic foot problems. http://www.nice.org.uk/guidance/NG19
- NICE NG18 (2015, updated 2023). Diabetes (type 1 and type 2) in children and young people. http://www.nice.org.uk/guidance/NG18
- NICE NG17 (2015, updated 2022). Type 1 diabetes in adults. http://www.nice.org.uk/guidance/NG17

iii. The indicators for diabetes are generally those which would be expected to be done, or checked, in an annual review. There is no requirement for the contractor to carry out all these items, but it is the contractor's responsibility to ensure that they have been done.

DM006 (based on NICE IND134)

DM006 Rationale

i. NICE guidelines[21],[22] recommend the use of an ACE-I (or ARBs) to slow the progression of renal disease in patients with diabetes with urine albumin: creatinine ratio (ACR) ≥3 mg/mmol. Trial evidence suggests that these are most effective when given in the maximum dose quoted in the BNF. NICE guidelines also recommend that SGLT2i should be offered or considered, depending on the level of ACR, in people with type 2 diabetes and renal disease.

ii. It is recommended that patients with a diagnosis of micro-albuminuria or proteinuria are commenced on an ACE-I or ARBs.

DM006 Reporting and verification

i. See indicator wording for requirement criteria.

DM012 (based on NICE IND81)

DM012 Rationale

i. Patients with diabetes are at high risk of foot complications that could lead to ulcer, amputation or death. Evaluation and risk classification on an annual basis are important for the detection of feet most at risk.

ii. The NICE guideline on diabetic foot problems[23] outlines foot risk classification and recommends at least annual reassessment.

iii. For the purposes of QOF, the clinical codes for 'moderate risk' are used to record the concept of 'increased risk'.

DM012 Reporting and verification

i. See indicator wording for requirement criteria.

[21] NICE NG17 (2015, updated 2022). Type 1 diabetes in adults. https://www.nice.org.uk/guidance/ng17
[22] NICE NG28 (2015, updated 2022). Type 2 diabetes in adults. https://www.nice.org.uk/guidance/ng28
[23] NICE NG19 (2015, updated 2019) Diabetic foot problems: prevention and management. http://www.nice.org.uk/guidance/NG19/

DM014 (based on NICE IND88)

DM014 Rationale

i. Diabetes is a long-term medical condition that is predominantly managed by the person with the diabetes and/or their carer as part of their daily life. Accordingly, understanding of diabetes and the acquisition of relevant skills for successful self-management play an important role in achieving optimal outcomes. These needs are not always adequately fulfilled by conventional clinical consultations. Structured educational (SE) programmes have been designed not only to improve people's knowledge and skills, but also to help motivate and sustain people with both type 1 and type 2 diabetes in taking control of their condition and in delivering effective self-management. Structured education programmes are supported by NICE guidance[24,25].

ii. The indicator requires that SE is offered to every person with diabetes and/or their carer from the time of diagnosis. An alternative education programme of equal standard may be offered to people unable or unwilling to participate in group education sessions.

iv. There are several accredited digital education programmes including the nationally commissioned Healthy Living with type 2 diabetes. Referral to this programme will also meet the criteria for this indicator and it is available to people with type 2 diabetes and their carers at any timeframe following diagnosis, supporting annual reinforcement.

iii. This indicator suggests referral to a programme within nine months of entry onto the diabetes register to be appropriate for people with type 1 or type 2 diabetes. A timeframe of nine months for this indicator has been set to take into account the differing expectations for referral into SE programmes from diagnosis for people with type 1 and type 2 diabetes.

DM014 Reporting and verification

i. See indicator wording for requirement criteria. For measurement purposes, nine months is defined as 279 days.

[24] NICE NG17 (2015, updated 2022). Type 1 diabetes in adults. https://www.nice.org.uk/guidance/ng17
[25] NICE NG28 (2015, updated 2022). Type 2 diabetes in adults. https://www.nice.org.uk/guidance/ng28

DM036 (based on NICE IND249)

DM036 Rationale

i. Lowering blood pressure in people with diabetes reduces the risk of developing micro and macrovascular complications.

ii. Applying universal BP targets to all people with diabetes may inadvertently lead to the potential for undertreatment in those with less complex need and overtreatment in those with complex needs and co-morbidity[26]. This indicator focuses upon blood pressure management in people with diabetes without moderate or severe frailty and thus aims to reduce potential undertreatment and support better control of biomedical targets in people with the greatest capacity to benefit.

iii. Contractors should note that the BP target in this indicator is higher than that recommended in NG17 for patients with type 1 diabetes aged 79 or under with ACR of 70 mg/mmol or more, where they should be aiming for under 130/80mmHg. Contractors should use their clinical judgement when setting individual blood pressure targets, particularly for people with advanced age, living with frailty or multimorbidity.

iv. This indicator was updated in 2024/25 to align with updated NICE guidance which recommends differential blood pressure control targets for readings taken with Home Blood Pressure Monitoring (HBPM) and readings taken in a clinical setting. A target for stage 1 hypertension of 140/90mmHg taken in a clinic corresponds to an HBPM target of 135/85 mmHg.

DM036 Reporting and verification

i. See indicator wording for requirement criteria.

DM020 (based on NICE IND179)

DM020 Rationale

i. Glycated haemoglobin (HbA1c) is commonly used to monitor glucose control as it provides a measure of average glycaemia over the preceding 8-12 weeks. Rising levels of HbA1c increase the risk of mortality and developing macrovascular and microvascular complications. However, applying universal target levels regardless of comorbidities may inadvertently lead to over-treatment, especially in older people with

[26] Kearney et al. Overtreatment and undertreatment: time to challenge our thinking. BJGP. 2019;67(633):442-443.

type 2 diabetes and people living with frailty.[27] This indicator allows for an individualised management approach that adjusts care according to an individual's frailty status. It aims to enable patients without moderate or severe frailty to benefit from tighter glycaemic control. Whilst the target in this indicator is higher than those presented in NICE guidelines[28, 29], this has been pragmatically selected as it represents the point at which people with type 2 diabetes should be considered for treatment intensification.

DM020 Reporting and verification

i. See indicator wording for requirement criteria.

DM021 (based on NICE IND180)

DM021 Rationale

i. This indicator allows for an individualised management approach that adjusts care according to an individual's frailty status. It aims to reduce complications and improve quality of life for people with moderate or severe frailty. NICE guidelines recommend that individualised HbA1c targets should be agreed with people with both type 1 and type 2 diabetes which consider factors such as their daily activities, aspirations, likelihood of complications, comorbidities, and occupation. Individual targets, even for people with moderate or severe frailty, should be lower than the level specified in this indicator. The target in this indicator has been pragmatically selected as a level that HbA1c should not go beyond to avoid people becoming symptomatic of hyperglycaemia.

DM021 Reporting and verification

i. See indicator wording for requirement criteria.

[27] Strain et al. Type 2 diabetes mellitus in older people: a brief statement of key principles of modern day management including the assessment of frailty. Diabetic medicine. 2018;35(7): 838-845.
[28] NICE NG17 (2015, updated 2022). Type 1 diabetes in adults. https://www.nice.org.uk/guidance/NG17
[29] NICE NG28 (2015, updated 2022). Type 2 diabetes in adults. https://www.nice.org.uk/guidance/NG28

DM034 (based on NICE IND275)

DM034 Rationale

i. Cardiovascular risk is elevated in people with type 1 and type 2 diabetes. The NICE guideline for cardiovascular disease risk assessment and lipid modification[30] recommends that people with type 1 diabetes are offered statin treatment for primary prevention when they are older than 40 years, or they have had diabetes for more than 10 years, or they have established nephropathy or other CVD risk factors. It also recommends that people with type 2 diabetes should be offered statin therapy if they have a 10% or greater 10-year risk of developing CVD, estimated using the QRISK3 assessment tool. The business rules for this indicator include clinical codes for QRISK, QRISK2, QRISK3, Framingham and Joint British Societies risk score.

ii. In 2023, the NICE guideline for cardiovascular disease: risk assessment and reduction, including lipid modification reinforced the recommendation of high-intensity statin treatment, for primary prevention (atorvastatin 20mg) and secondary prevention (atorvastatin 80mg). This indicator was updated in 2024/25 to reflect the potential for use of other lipid lowering therapies where a statin is declined or clinically unsuitable.

DM034 Reporting and verification

i. See indicator wording for requirement criteria.

ii. People with type 2 diabetes who have a less than 10% 10-year risk of developing CVD recorded in the preceding 3 years will be excluded from the denominator for this indicator.

DM035 (based on NICE IND276)

DM035 Rationale

i. The NICE guideline for cardiovascular disease risk assessment and lipid modification[31] recommends that lipid lowering treatments should be offered for the secondary prevention of CVD. For most people, this will include high intensity statin therapy, which has been shown to lower levels of low-density lipoprotein (LDL) cholesterol and is associated with a reduction in MI, coronary heart disease and

[30] NICE NG238 (2023) Cardiovascular disease: risk assessment and reduction, including lipid modification. https://www.nice.org.uk/guidance/ng238

[31] NICE NG238 (2023) Cardiovascular disease: risk assessment and reduction, including lipid modification. https://www.nice.org.uk/guidance/ng238

ethnicity is recorded as White) or preceding 24 months for all other patients		
MH012. The percentage of patients with schizophrenia, bipolar affective disorder and other psychoses who have a record of blood glucose or HbA1c in the preceding 12 months	7	50-90%

MH – rationale for inclusion of indicator set

i. This indicator set reflects the complexity of mental health problems, and the complex mix of physical, psychological and social issues that present to GPs.

ii. For many patients with mental health problems, the most important aspects of care quality relate to the interpersonal skills of the doctor, the time given in consultations and the opportunity to discuss a range of management options.

iii. An important aim of the indicator set is to help address a major health inequality experienced by people with severe mental illness (SMI) reflected in a reduced life expectancy of around 15-20 years[52]. This disparity is largely due to preventable physical illnesses. People with SMI develop chronic physical health conditions at a younger age than people without SMI. These chronic conditions include obesity, asthma, diabetes, chronic obstructive pulmonary disease, coronary heart disease, stroke, heart failure and liver disease. People with SMI are at increased risk of developing more than one of these chronic conditions[53].

iv. NICE CG178[54] recommends that primary care utilise registers to monitor the physical health of patients with psychosis or schizophrenia. The health check should be comprehensive, focusing on physical health problems that are common in people with psychosis and schizophrenia. NICE recommends health checks should include the following:

[52] Public Health England (2018) Severe mental illness (SMI) and physical health inequalities: briefing. https://www.gov.uk/government/publications/severe-mental-illness-smi-physical-health-inequalities/severe-mental-illness-and-physical-health-inequalities-briefing

[53] Office for Health Improvement & Disparities (2023) Premature mortality in adults with severe mental illness (SMI) https://www.gov.uk/government/publications/premature-mortality-in-adults-with-severe-mental-illness/premature-mortality-in-adults-with-severe-mental-illness-smi

[54] NICE CG178 (2014) Psychosis and schizophrenia in adults. http://www.nice.org.uk/guidance/CG178

- weight (plotted on a chart)
- waist circumference
- pulse and blood pressure
- fasting blood glucose or glycosylated haemoglobin (HbA1c)
- blood lipid profile and prolactin levels
- assessment of any movement disorders
- assessment of nutritional status, diet and level of physical activity

v. NICE CG185[55] recommends that patients with bipolar affective disorder have a physical health review, normally in primary care, performed at least annually, including:

- weight or BMI, diet, nutritional status and level of physical activity
- cardiovascular status, including pulse and blood pressure
- metabolic status, including glycosylated haemoglobin (HbA1c) and blood lipid profile
- liver function
- renal and thyroid function, and calcium levels, for people taking long-term lithium

vi. A key focus of the SMI annual health check is to identify those individuals with risk factors (including metabolic syndrome) for cardiovascular disease (CVD) and type 2 diabetes, given these disorders are 2-3x more common than in the general population and are major contributors to the life expectancy gap.[56]. Clustering of risk factors (obesity, hypertension, glucose and lipid disturbances) are caused by the accumulative effect of adverse metabolic effects from psychotropic medication and poor health behaviours (poor diet, physical inactivity).[57] Risk is further compounded by substantially higher rates of smoking than in the general population.[58] A full annual check and supported signposting to appropriate interventions is therefore important for reducing risk.

[55] NICE CG185 (2014, updated 2023) Bipolar disorder: assessment and management. http://www.nice.org.uk/guidance/CG185
[56] Ma R, Romano E, Ashworth M, Yadegarfar ME, Dregan A, Ronaldson A, de Oliveira C, Jacobs R, Stewart R, Stubbs B. Multimorbidity clusters among people with serious mental illness: a representative primary and secondary data linkage cohort study. Psychol Med. 2023 Jul;53(10):4333-4344. doi: 10.1017/S003329172200109X.)
[57] Vancampfort D, Stubbs B, Mitchell AJ, De Hert M, Wampers M, Ward PB, Rosenbaum S, Correll CU. Risk of metabolic syndrome and its components in people with schizophrenia and related psychotic disorders, bipolar disorder and major depressive disorder: a systematic review and meta-analysis. World Psychiatry. 2015 Oct;14(3):339-47
[58] Szatkowski L, McNeill A. Diverging trends in smoking behaviors according to mental health status. Nicotine Tob Res 2014; 17: 356–60.

ii. Verification – Commissioners may require contractors to randomly select a number of care plans to ensure that they are being reviewed annually and updated where necessary.

MH003, MH006, MH007, M011 and MH012 (based on NICE IND84, IND83, IND82, IND158, IND159 respectively)

MH003, MH006, MH007, M011 and MH012 Rationale

i. NICE guidance[62,63] recommends annual monitoring of blood pressure for people with bipolar disorder, psychosis or schizophrenia. A prospective record linkage study of the mortality of a community cohort of 370 patients with schizophrenia found that the increased mortality risk is probably life-long and it suggested that the cardiovascular mortality of people with schizophrenia has increased over the past 25 years relative to the general population[64]. The NICE guideline on bipolar disorder also states that the standardised mortality ratio for cardiovascular death may be twice that of the general population but appears to be reduced if patients adhere to long-term medication.

iii. While disturbances such as impaired glucose tolerance and diabetes, hypertension, and dyslipidaemia tend to emerge around middle age in the general population, in people with SMI they may be detectable from as early as the first presentation. This helps to explain why cardiovascular risk prediction tools developed primarily for the general population underestimate risk in young people with SMI[65],[66].

iv. Recording (and treating) cardiovascular risk factors is therefore very important for patients with a severe mental illness.

v. MH007 incentivises delivering the requirement to record alcohol intake as part of the physical check. Alcohol and other substance misuse by people with schizophrenia is increasingly recognised as a major problem, both in terms of its prevalence and its clinical and social effects[67]. The National Psychiatric Morbidity Survey in England

[62] NICE CG178 (2014). Psychosis and schizophrenia in adults. http://www.nice.org.uk/guidance/CG178
[63] NICE CG185 (2014, updated 2020) Bipolar disorder. https://www.nice.org.uk/guidance/cg185
[64] Brown S, Kim M, Mitchell C et al. 25 year mortality of a community cohort with schizophrenia. BJP. 2010. 196: 116-21.
[65] Perry BI, McIntosh G, Weich S, et al. The association between first-episode psychosis and abnormal glycaemic control: systematic review and meta-analysis. Lancet Psychiatry 2016; 3(11): 1049–1058.
[66] Perry BI, Upthegrove R, Crawford O, et al. Cardiometabolic risk prediction algorithms for young people with psychosis: a systematic review and exploratory analysis. Acta Psychiatrica Scand 2020; 142(3): 215–232.
[67] RCP Research and Training Unit. Banerjee S, Clancy C, Crome I, editors. Co-existing problems of mental disorder and substance misuse (dual diagnosis). 2001. Information manual.

found that 16% of people with schizophrenia were drinking above the lower risk consumption levels (14 units) of alcohol[68,69]. Bipolar affective disorder is also highly co-morbid with alcohol and other substance abuse.

vi. NICE guidance[70,71] recommends annual monitoring of blood glucose or HbA1c for people with bipolar disorder, psychosis or schizophrenia. Diabetes is 2–3 times more common among people with SMI than the general population[72] and antipsychotic medication can be diabetogenic[73]. People with SMI are more likely to develop type 2 diabetes earlier than the general population, frequently in the fourth and fifth decades.

MH003, MH006, MH007, M011 and MH012 Reporting and verification

i. See indicator wording for requirement criteria.

ii. Within the business rules currently being prescribed an antipsychotic medication is defined as a prescription in the preceding 6 months; pre-existing cardiovascular conditions are defined as CHD, diabetes, stroke, peripheral arterial disease and chronic kidney disease; being a current smoker is defined as a patient whose notes record smoking status in the preceding 12 months and being overweight is defined as latest BMI of ≥ 23 kg/m2 or ≥25 kg/m2 if ethnicity is recorded as white.

iii. Patients who have a diagnosis of diabetes will be excluded from MH012.

3.12 Non-diabetic hyperglycaemia (NDH)

Indicator	Points	Thresholds
Records		

[68] Meltzer H, Gill B, Pettigrew M et al. OCPS Survey of Psychiatric Morbidity in GB. Report 3: Economic activity and social functioning of adults with psychiatric disorders. 1996.
[69] Farrell M, Howes S, Taylor C et al. Substance misuse and psychiatric co-morbidity: an overview of the OCPS National Psychiatric Morbidity Survey. Addictive behaviours 23: 909-18. 1998.
[70] NICE CG178 (2014). Psychosis and schizophrenia in adults. http://www.nice.org.uk/guidance/CG178
[71] NICE CG185 (2014, updated 2020) Bipolar disorder. https://www.nice.org.uk/guidance/cg185
[72] Holt, R. I., & Mitchell, A. J. (2015). Diabetes mellitus and severe mental illness: mechanisms and clinical implications. Nat Rev Endocrinol, 11(2), 79-89. https://doi.org/10.1038/nrendo.2014.203
[73] Smith M, Hopkins D, Peveler RC, et al. (2008) First-v. second-generation antipsychotics and risk for diabetes in schizophrenia: systematic review and meta-analysis. Br J Psychiatry 192(6):406–411.

NDH002. The percentage of patients with non-diabetic hyperglycaemia who have had an HbA1c or fasting blood glucose performed in the preceding 12 months	18	50–90%

NDH – rationale for inclusion of indicator set

vii. NDH (also known as prediabetes) is defined as an HbA1c of 42-47mmol/mol or a fasting plasma glucose (FPG) of 5.5-6.9mmol/l[74]. There were more than 3.7 million people with NDH in England in 2023/24, representing an increase of 530,000 people compared with the previous year.

i. The NHS has invested significantly in behavioural interventions for those with NDH in order to prevent and delay the onset of type 2 diabetes. The Healthier You: NHS Diabetes Prevention Programme (NHS DPP) is the largest undertaking of its kind in the world and over 840,000 people have participated since its introduction in 2016. An independent evaluation of the programme has demonstrated its effectiveness, with programme completion (defined as attending >60% of sessions) associated with a relative risk reduction of 37% for the development of type 2 diabetes.

ii. The NHS DPP is available across the whole of England, and GP practices can refer patients aged 18 or over with a blood test result demonstrating NDH in the 12 months prior to referral. Individuals with a history of Gestational Diabetes Mellitus (GDM) are also eligible and have an additional route of accessing the programme through self-referral. Participants of the NHS DPP must not be pregnant, have ever been diagnosed with type 2 diabetes, be recorded as living with moderate/severe frailty, have an active eating disorder or have had bariatric surgery within the previous 2 years.

NDH002 (based on NICE IND172)

NDH002 Rationale

i. NICE Guidance (PH38[75]) recommends that everyone with NDH is offered an annual blood test to check for progression to Type 2 diabetes. Despite this, there is wide variation in the monitoring of people with NDH.

[74] NICE PH38 (2012, updated 2017) Type 2 diabetes: prevention in people at high risk http://www.nice.org.uk/guidance/ph38

[75] NICE PH38 (2012, updated 2017) Type 2 diabetes: prevention in people at high risk http://www.nice.org.uk/guidance/ph38

ii. The aim of this indicator is to promote early identification if people progress from having NDH to type 2 diabetes, as early recognition and management of diabetes is associated with improved long-term outcomes. Criteria for diagnosing diabetes are discussed in the diabetes section of this guidance.

NDH002 Reporting and verification

i. See indicator wording for requirement criteria.

ii. The register for the purpose of calculating the APDF is defined as all patients aged 18 or over with a record of non-diabetic hyperglycaemia or pre-diabetes, which has not been superseded by a diagnosis of diabetes recorded prior to the beginning of the financial year.

4. Public health domain

4.1 Blood pressure (BP)

Indicator	Points	Thresholds
BP002. The percentage of patients aged 45 or over who have a record of blood pressure in the preceding 5 years	15	50–90%

BP002 (based on NICE IND112)

BP002 Rationale

i. Detecting elevated blood pressure and, where indicated, treating it, is known to be an effective health intervention. Raised blood pressure is common if it is measured on a single occasion but with repeated measurement blood pressure tends to drop. NICE guideline recommendations for the diagnosis and treatment of hypertension[76] are to be followed by practitioners when deciding on whether to treat raised blood pressure.

ii. The age limit of aged 45 or over has been chosen as the vast majority of patients develop hypertension after this age. The age range 45 or over, coupled with a five-year reference period is in line with the NHS Health Checks Scheme, which starts at 40 years old. It is also to align the indicator more closely with the vascular checks

[76] NICE NG136 (2019, updated 2023) Hypertension in adults http://www.nice.org.uk/guidance/ng136

programme and the cost-effectiveness modelling undertaken to support that programme.

iii. It is anticipated that contractors will opportunistically check blood pressures in all adult patients.

BP002 Reporting and verification

i. See indicator wording for requirement criteria.

ii. Generally, personalised care adjustment criteria (see Section 6) do not apply to this indicator. However, practices are able to remove patients from the denominator where the patient declines to accept offered care.

4.2 Smoking (SMOK)

Indicator	Points	Thresholds
Records		
SMOK002. The percentage of patients with any or any combination of the following conditions: CHD, PAD, stroke or TIA, hypertension, diabetes, COPD, CKD, asthma, schizophrenia, bipolar affective disorder or other psychoses whose notes record smoking status in the preceding 12 months	25	50–90%
Ongoing management		
SMOK004. The percentage of patients aged 15 or over who are recorded as current smokers who have a record of an offer of support and treatment within the preceding 24 months	12	40–90%

CG	Clinical guideline (NICE)
CHD	Coronary Heart Disease
$CHADS_2$	Congestive (HF) Hypertension Age (75 or over) Diabetes Stroke
CHA_2DS_2-VASc	Congestive (HF) Hypertension Age (75 or over) Diabetes Stroke (prior stroke) Vascular Disease (peripheral artery disease) Age (65–74 years) Sex Category (i.e. female)
CKD	Chronic Kidney Disease
CMO	Chief Medical Officer
COPD	Chronic Obstructive Pulmonary Disease
CPA	Care Programme Approach
CQRS	Calculating Quality Reporting Service
CRP	C-Reactive Protein
CS	Cervical Screening
CVD	Cardiovascular Disease
CVD-PP	CVD Primary Prevention
DEM	Dementia
DEP	Depression
DM	Diabetes Mellitus
DMARD	Disease Modifying Anti-Rheumatic Drugs
DXA	Dual-Energy X-ray Absorptiometry
ED	Erectile Dysfunction
eGFR	Estimated Glomerular Filtration Rate
EOLC	End of Life Care
EP	Epilepsy
ES	Enhanced Service
ESR	Erythrocyte Sedimentation Rate
FEV_1	Forced Expiratory Volume in One Second
FVC	Forced Vital Capacity
GFR	Glomerular Filtration Rate
GMC	General Medical Council
GMS	General Medical Services

GOLD	The Global Initiative for Chronic Obstructive Lung Disease
GP	General Practitioner
GPC England	General Practitioners Committee England
GPES	General Practice Extraction Service
GSF	Gold Standards Framework
HbA1c	Glycated Haemoglobin
HBPM	Home Blood Pressure Monitoring
HF	Heart Failure
HYP	Hypertension
IFCC	International Federation of Clinical Chemistry and Laboratory Medicine
IQ	Intelligence Quotient
JCVI	Joint Committee on Vaccination and Immunisation
LD	Learning Disabilities
LDL	Low Density Lipoprotein
LVSD	Left Ventricular Systolic Dysfunction
MDT	Multi-disciplinary team
MH	Mental Health
MI	Myocardial Infarction
mmHg	Millimetres of Mercury
mmol/l	Millimoles per Litre
MRC	Medical Research Council
NCSI	National Cancer Survivorship Initiative
NDH	Non-Diabetic Hyperglycaemia
NG	NICE guideline
NHS	National Health Service
NICE	National Institute for Health and Care Excellence
NRT	Nicotine Replacement Therapy
NSF	National Service Framework
OB	Obesity
OGTT	Oral Glucose Tolerance Test